



STRATEGY & OPERATIONS

Vytalix Customer Segments, Icp And Target-Account Archetypes

Vytalix company-build documentation · reference
06_CUSTOMER_SEGMENTS

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR
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Owner: Chief Strategy Officer · **Status:** PROPOSED v1 · **Date:** 8 September 2026 · **Governing file:** [/FACTS_BASE.md](#) (KNOWN / VERIFIED / ASSUMPTION / ESTIMATE / PROPOSED / TO VALIDATE apply throughout). Vytalix has no customers, pilots, partners or references in any segment today (KNOWN). Every budget, sales-cycle and price figure below is an ESTIMATE from desk research and sector experience unless a URL is given; each must be tested in discovery calls before it is used in a proposal. Prices reference the service catalogue in [/docs/01_BUSINESS_MODEL.md](#) §5 (codes C1–C8, A1–A2, T1, E1).

Executive view

- 1. **Eighteen segments were examined (13 B2B, 5 B2C); only four can pay Vytalix within 90 days:** health-tech companies (UK and inbound), NGOs and development programmes with live budget lines, private healthcare groups/clinics, and universities/accelerators buying training. NHS trusts, ICBs, pharma, insurers and governments are 6–18-month cycles that a solo, unincorporated founder cannot close this year — they are relationship-building segments in 2026 and revenue segments in 2027–28.
- 2. **The ideal customer profile (ICP) for the services business is a UK or inbound digital-health company, pre-seed to Series A, with an NHS or African go-to-market problem and a founder who decides in a week.** It scores 89/100 on the ICP model in §3 versus 46/100 for NHS trusts, because it combines short cycle, founder-to-founder access, a fee (£7.5k–£22k, ESTIMATE) that needs no procurement, and direct learning for MaternaLink (in development).
- 3. **MaternaLink's buyers and MaternaLink's users are different segments with different economics.** Buyers are trusts, private maternity units, state ministries via donors, NGOs and (later) insurers/HMOs; users are expectant mothers, families, midwives and community health workers. B2C segments are therefore treated as *design and adoption* segments, not revenue segments, except for E-Learning where individual professionals pay £49–£299 per course (ESTIMATE).
- 4. **Ten target-account archetypes (§4) — not ten named companies — define the first 90 days of outbound.** Each archetype has a trigger that can be observed publicly (a funding announcement, a CQC rating, a tender, a grant call), an entry offer, and a decision-maker title. Named accounts are populated in the CRM only from public sources and only after the archetype is validated by three conversations.
- 5. **The Ekiti/Nigeria government segment is deliberately ranked low for 2026 revenue** despite being the only live conversation: the state has a funded incumbent (mDoc / MSD for Mothers, VERIFIED), pricing is unvalidated, and government payment risk is high. It stays in the plan as a partnership and evidence segment (see [/docs/12_PARTNERSHIP_STRATEGY.md](#)), with NGO-funded cohorts as the commercial route.

0. Method and definitions

Term	Meaning in this document
Segment	A group of organisations or people who share a buying problem, a decision-maker type and a budget source.
Pillar	Which Vytalix division sells to the segment: Advisory (founder-led days/retainers), Consulting (fixed-price sprints C1–C8), Health Solutions (MaternaLink, in development — pilots only), E-Learning (courses, licences).
Budget (ESTIMATE)	The realistic first-engagement value Vytalix could win from a single account in the next 12–24 months; not the segment's total spend.
Sales cycle	First contact to signed order, ESTIMATE, assuming no prior relationship.
Horizon	Now (invoice within 90 days of incorporation); 6m ; 12m ; 24m+ .
Objections	The three most likely reasons the buyer says no, phrased as the buyer would say them; responses live in /docs/13_SALES_ENGINE.md §15.

Segment ranking uses the ICP model in §3 (six weighted criteria). All scores are desk ESTIMATES to be re-scored after the first 40 discovery conversations.

1. B2B segments

1.1 Hospitals and NHS trusts (maternity services)

Attribute	Detail
Who	~120 NHS trusts in England provide maternity services (ESTIMATE; TO VALIDATE exact count), delivering ~568k births in England in 2024 (ONS, https://www.ons.gov.uk/peoplepopulationandcommunity/birthsdeathsandmarriages/livebirths/bulletins/birthsummarytablesenglandandwales/2024refreshedpopulations); plus Welsh/Scottish/NI boards and trusts. Sub-types: large teaching trusts (≥6,000 births), district general hospitals (3,000–5,000), trusts named in the 2025–26 national maternity investigation (BMJ Group, https://bmjgroup.com/maternity-care-14-nhs-trusts-at-centre-of-investigation-named/), trusts with CQC "requires improvement"/"inadequate" maternity ratings.
Pain points	Communication and handover failures identified by inquiries; MBRACE-UK disparity findings (Black women more than twice as likely to die — NHS Providers summary, https://nhsproviders.org/resources/summary-of-the-2025-mbrace-uk-saving-lives-improving-mothers-care-report); CNST Maternity Incentive Scheme evidence burden; MSDS data-quality; maternity EPR re-procurement (Euroking replacements 2026–28, see /docs/05_MARKET_STRATEGY.md §1.1); midwifery workforce shortages; complaints volumes; national maternity dashboard scrutiny.
Buying triggers (observable)	CQC inspection report published; inclusion in the national investigation; new Director of Midwifery appointed; maternity EPR tender notice; LMNS equity action plan due; a serious-incident review published; a Health Innovation Network programme call.
Decision maker	Director/Head of Midwifery (owner); Maternity Digital Lead / digital midwife (champion); CNIO and CIO (assurance); Clinical Safety Officer (DCB0160 gate); Divisional Director of Operations (budget); procurement (route). For consulting under threshold: divisional director + head of midwifery.
Budget (ESTIMATE)	Consulting: £15k–£30k direct award under local thresholds (C3 £24k, C8 £28k); improvement budgets often held at LMNS/ICB level. Product: £0 for a design partnership; paid pilot £20k–£60k; licence £40k–£120k/year per trust (from /docs/04_MATERNALINK_PRODUCT_STRATEGY.md §9).
Sales cycle	Consulting 3–9 months (relationship + procurement); design partnership 6–12 months; paid product 9–18 months.

Attribute	Detail
Objections	"Are you DTAC-compliant and do you have a clinical safety case?" · "We already have BadgerNet/K2 — why another system?" · "We have no budget; can the Health Innovation Network fund it?"
Vytalix proposition (pillar)	Consulting first: Digital Maternity Readiness Review (C3) and Health Inequalities & Maternity Outcomes Review (C8), vendor-neutral. Health Solutions later: MaternaLink (in development) as the woman-facing communication and handover layer beside the EPR — never an EPR — offered as an unpaid design partnership under DCB0160, then a paid pilot.
Acquisition strategy	Land through people, not tenders: a clinical adviser (obstetrician/senior midwife) opens doors; Health Innovation Network programmes for introductions; LMNS meetings; a sourced "State of digital maternal care" report as the reason to talk. Publish two case-free thought-leadership pieces per month on handover and listening. Sub-contract to an established consultancy for the first NHS reference.
Horizon	Consulting 12m ; product 24m+ .

1.2 Clinics (private maternity units, GP federations, community and independent midwives)

Attribute	Detail
Who	London private maternity units (e.g., The Portland, the Kensington Wing, the Lindo Wing — named as public institutions only; fit TO VALIDATE); private obstetric practices; independent midwives (~100–200 practising in the UK, ESTIMATE); GP federations and primary-care networks with maternity/women's-health hubs; in Nigeria/Kenya, private maternity clinics and faith-based hospitals.
Pain points	Premium clients expect continuous, personalised communication; small teams have no digital budget for enterprise EPRs; independent midwives rely on WhatsApp and paper; private units need differentiation versus NHS; in Africa, patient follow-up and ANC defaulting hit revenue directly.
Buying triggers	New service launch or refurbishment; patient-experience complaints; CQC inspection; competitor launches an app; a partner obstetrician asks for it.
Decision maker	Hospital director / general manager (private units); lead consultant obstetrician (clinical sponsor); practice owner (independent midwives); medical director (African private hospitals).
Budget (ESTIMATE)	Consulting: £5k–£14k (C1 discovery sprint or a shortened version). Product: per-woman-per-episode licence £30–£80 (UK); £3–£12 per enrolled pregnancy (Africa, from /docs/04_MATERNALINK_PRODUCT_STRATEGY.md §9). First-year account value £5k–£25k.
Sales cycle	4–12 weeks (private UK); 2–4 months (African private hospitals).
Objections	"Our patients already have my mobile number." · "Is it secure/GDPR-compliant?" · "Can it work with our existing system?"
Vytalix proposition (pillar)	Health Solutions: MaternaLink (in development) as a fast-credibility pilot market with fewer IG barriers; Consulting: C1 discovery sprint for a service line launching digital maternity care.
Acquisition strategy	Direct outreach to hospital directors and lead obstetricians; independent-midwife associations and networks (no relationship claimed); a paid pilot bundle (£10k minimum per /docs/13_SALES_ENGINE.md §16) with the safety artefacts included.
Horizon	Consulting 6m ; product 12m (after IP resolved, prototype audited, DPIA and safety case in place).

1.3 Healthcare groups (private hospital groups, multi-site providers)

Attribute	Detail
Who	UK private hospital groups (e.g., HCA UK, Spire, Nuffield, Circle — institutions only; maternity footprint TO VALIDATE); community-service providers holding NHS contracts; in Nigeria/Kenya, multi-site private providers and hospital chains (e.g., Lagoon, Reddington, AAR, Penda — institutions only, TO VALIDATE).
Pain points	Group-wide digital strategy versus site autonomy; patient-experience metrics across sites; NHS-contract requirements (DSPT, DTAC) for private providers of NHS care; in Africa, referral leakage and follow-up.
Buying triggers	Group strategy refresh; new CDO/CIO; acquisition of a maternity site; NHS contract renewal requiring digital evidence; board-level women's-health initiative.
Decision maker	Group Chief Digital/Strategy Officer; Group Medical Director; divisional MD for women's health; procurement.
Budget (ESTIMATE)	Consulting: £14k–£28k per sprint (C1, C3); enterprise MSA £50k–£300k/year over time (from /docs/01_BUSINESS_MODEL.md §2 #8).
Sales cycle	3–6 months.
Objections	"We use a Big Four / a specialist consultancy." · "Show us references at our scale." · "Why not build it in-house?"
Vytalix proposition (pillar)	Consulting (C1, C3) with a founder-delivered, fixed-price boutique promise; Advisory (fractional strategy officer, offer #9).
Acquisition strategy	LinkedIn outbound to group digital leaders; speak at private-healthcare and digital-health events (see /docs/14_MARKETING_AND_CONTENT_ENGINE.md); referrals from health-tech clients who sell into the same groups.
Horizon	6m–12m .

1.4 Pharmaceutical companies (medical affairs, patient support, global health / CSR units)

Attribute	Detail
Who	Global pharma with women's-health portfolios or maternal-health CSR programmes (MSD for Mothers is a \$500m initiative, per Devex, https://www.devex.com/organizations/msd-for-mothers-135550 , and funds mDoc's Digital Mom Project in Lagos and Ekiti — VERIFIED per FACTS_BASE); UK affiliates' medical-affairs and market-access teams; generics/contraception and anaemia-product companies active in Nigeria/Kenya.
Pain points	Demonstrating real-world impact of programmes; patient-support programme adherence; health-inequality commitments; needing local implementing capacity in African markets; ABPI Code constraints on patient-facing activity.

Attribute	Detail
Buying triggers	New CSR/global-health funding cycle; a product launch in maternal health (e.g., anaemia, hypertension in pregnancy); an RFP for programme evaluation; an NHS partnership initiative.
Decision maker	Global health / CSR programme director; medical affairs lead; market-access director; procurement (vendor onboarding is slow).
Budget (ESTIMATE)	Consulting/evaluation: £26k–£80k (offer #10 Programme Evaluation Lite; landscape studies); programme funding for MaternaLink (in development) cohorts £50k–£250k if won via a call (TO VALIDATE).
Sales cycle	6–12 months; vendor onboarding adds 1–3 months.
Objections	"We fund NGOs, not companies." · "Conflict with our existing grantee (mDoc)." · "You have no evaluation track record."
Vytalix proposition (pillar)	Consulting (evaluation, landscape, market-entry scans); Health Solutions only via funded programmes with an NGO lead; E-Learning co-branded health-worker modules (24m+).
Acquisition strategy	Apply to published calls; partner with an NGO as prime; publish the maternal digital-care report to be visible to programme officers; ABPI-compliant positioning (no promotional content).
Horizon	12m–24m.

1.5 Insurers, HMOs and benefits platforms

Attribute	Detail
Who	UK private medical insurers (limited maternity cover — TO VALIDATE current products) and employee-benefits platforms; Nigerian HMOs and state health insurance agencies (e.g., AXA Mansard, Reliance HMO, Hygeia — institutions only, interest TO VALIDATE); Kenya's Social Health Authority and private medical insurers.
Pain points	Maternity claims cost and complications; ANC adherence and facility-delivery rates (Africa); member engagement; differentiation of benefits packages; fraud and utilisation data quality.
Buying triggers	Product redesign cycle; regulator pressure on maternal outcomes; a corporate client asking for a maternity benefit; SHA/NHIA scheme changes.
Decision maker	Head of product / benefits design; medical director; chief actuary (evidence gate); partnerships lead.
Budget (ESTIMATE)	Engagement fee £3–£12 per enrolled pregnancy per year (Africa); £150–£300 per supported pregnancy or £2–£6 PEPM via benefits platforms (UK) — from /docs/04_MATERNALINK_PRODUCT_STRATEGY.md \$9. Consulting on women's-health programme design £5k–£15k.
Sales cycle	6–12 months; actuarial evidence required.
Objections	"Prove the effect on claims." · "Data-sharing and consent — how?" · "We already work with a telehealth provider."
Vytalix proposition (pillar)	Health Solutions (adherence dashboards, aggregate metrics only, no underwriting or risk scoring); Consulting (women's-health benefit design).
Acquisition strategy	Not before pilot evidence exists; until then, discovery interviews only (five per market) to shape v2 features.
Horizon	24m+ (product); consulting opportunistic.

1.6 Governments — United Kingdom (NHS England/DHSC, ICBs/LMNS, local authorities, devolved nations)

Attribute	Detail
Who	42 ICBs each hosting a Local Maternity and Neonatal System (NHS England, https://www.england.nhs.uk/long-read/three-year-delivery-plan-for-maternity-and-neonatal-services/); NHS England (being absorbed into DHSC — TO VALIDATE status); local-authority public-health teams (maternal and infant health, health visiting); Welsh, Scottish and NI national digital programmes.
Pain points	Delivering the maternity delivery plan themes (listening, workforce, safety culture, standards); health-inequality action plans; ICB running-cost cuts (TO VALIDATE); MSDS data completeness; evaluating funded innovations.
Buying triggers	Delivery-plan reporting deadlines; a national inquiry recommendation; LMNS equity strategy refresh; a funded programme (e.g., a Health Innovation Network maternity call) needing delivery capacity.
Decision maker	LMNS programme director; ICB Director of Nursing/Midwifery; health-inequalities lead; public-health consultant (local authority); procurement.
Budget (ESTIMATE)	Consulting £25k–£150k per contract via framework or direct award under threshold (thresholds TO VALIDATE; /docs/01_BUSINESS_MODEL.md \$2 #6). Product: usually commissioned by trusts, not ICBs.
Sales cycle	3–9 months (consulting); reorganisation noise through 2026–27 lengthens it.
Objections	"You need to be on a framework." · "We are restructuring; no new commitments." · "Bigger firms have NHS references."
Vytalix proposition (pillar)	Consulting (C8 inequalities review; C2 business case; LMNS digital strategy; MSDS data-quality work); Advisory (fractional programme leadership).
Acquisition strategy	Sub-contract to a framework-listed prime for the first two engagements; register on G-Cloud/CCS frameworks once incorporated (TO VALIDATE routes); attend LMNS and MNVP forums with the clinical adviser.
Horizon	12m.

1.7 Governments — African (state ministries of health, primary-health-care agencies, national bodies)

Attribute	Detail
Who	Nigeria: 36 state MoHs + FCT and State Primary Health Care Development Agencies, federal FMOH/NPHCDA/NHIA; Kenya: 47 county health departments and the national MoH/SHA; Ghana Health Service; Rwanda MoH/RBC. The only live conversation is Ekiti (status unknown; proposal by David Agunede, March 2026 — VERIFIED as a proposal only). Ekiti already runs mDoc's Digital Mom Project with ~24,000 women onboarded (VERIFIED per FACTS_BASE).
Pain points	Highest maternal mortality burden globally (Nigeria MMR 993/100k in 2023, World Bank series, https://data.worldbank.org/indicator/SH.STA.MMRT?locations=NG); delays in seeking, reaching and receiving care; CHEW supervision; referral coordination; data reporting to DHIS2; donor-dependence for digital programmes; procurement and payment delays.
Buying triggers	New commissioner/governor priorities; a donor programme (MSD for Mothers, GFF, Gates, UNFPA) opening a state window; a national digital-health policy update; a maternal-mortality review with public pressure.
Decision maker	Commissioner for Health (political owner); Permanent Secretary and Director of Public Health/Family Health (technical); SPHCDA executive secretary (primary care); state ICT/digital-health lead; donor programme officer (the real budget holder in most cases).
Budget (ESTIMATE)	Advisory/programme design £15k–£80k (donor-funded); a credible Phase-1 programme £0.4m–£0.6m/year including implementation (re-based from the Ekiti proposal in /docs/04_MATERNALINK_PRODUCT_STRATEGY.md §9); platform £6–£40/woman/year. The £300/woman/year and £4.5m–£36m figures in the Ekiti proposal are UNVALIDATED (FACTS_BASE A5) and are not used.
Sales cycle	9–24 months; MoU → pilot agreement → contract; payment risk high; political cycles (Nigeria state elections).
Objections	"We already have a programme (mDoc)." · "Is your company registered here / where is the data hosted?" · "Who is funding this — we have no budget line."
Vytalix proposition (pillar)	Advisory (digital maternal-health programme design, pilot scoping, evaluation) now; Health Solutions (MaternaLink, in development) only as a partner-aware, LGA-scoped pilot with a local implementing partner, offered as a complement to any incumbent.
Acquisition strategy	Resolve Ekiti status in one call; one complementarity conversation with mDoc; score alternative states (Oyo, Kano lead in /docs/05_MARKET_STRATEGY.md §2.1); enter through NGO/donor cohorts rather than state budget lines; recruit a Nigerian health-system adviser (profile 4 in /docs/02_GROUP_STRUCTURE.md §4.2).
Horizon	Advisory 12m ; product 24m+ .

1.8 NGOs and development organisations (implementing partners, donors' programmes)

Attribute	Detail
Who	International NGOs with maternal-health portfolios (Jhpiego, PATH, MSI Reproductive Choices, Population Services International); national NGOs (Society for Family Health, Wellbeing Foundation Africa — institutions only); faith-based provider networks; UN agencies' country programmes (UNFPA, UNICEF); multilaterals (GFF/World Bank). No relationship exists with any (KNOWN).
Pain points	Donor reporting (logframes, DHIS2); CHEW supervision and defaulter tracing; cohort retention; evaluation rigour; sustainability after grant end; procurement of digital tools that outlive projects.
Buying triggers	A new grant award with a digital component; a call for proposals requiring a technology partner; an evaluation deadline; a programme re-design.
Decision maker	Country director (sponsor); programme manager/technical lead (owner); M&E lead (evidence gate); grants/compliance officer (procurement rules).
Budget (ESTIMATE)	Programme licence £8k–£60k/year per cohort (from /docs/04_MATERNALINK_PRODUCT_STRATEGY.md §9); evaluation/design consulting £10k–£50k (£4 in /docs/13_SALES_ENGINE.md); joint grant bids £50k–£500k.
Sales cycle	3–9 months, tied to grant cycles.
Objections	"Our donor prefers open-source/global-goods platforms." · "You are a company, not a nonprofit." · "Who maintains it after the grant?"
Vytalix proposition (pillar)	Consulting (programme design, evaluation lite, Africa partner scans); Health Solutions (programme licence with DHIS2/logframe exports, CHEW defaulter-tracing, data ownership to the NGO/state); joint grant applications.
Acquisition strategy	Map published grant calls quarterly; approach M&E leads with an evaluation offer; attend Nigeria Health Watch-type convenings (no relationship claimed); offer co-authorship of evaluations.
Horizon	Consulting 6m ; product 12m–18m .

1.9 Charities and community organisations (UK maternal-health and advocacy)

Attribute	Detail
Who	UK maternal-health charities and research funders (Tommy's, Wellbeing of Women, NCT, Best Beginnings, Birthrights, Five X More, Maternity Action, Sands, Bliss — institutions only; no relationship claimed); Maternity and Neonatal Voices Partnerships.
Pain points	Reaching women who are not reached by NHS channels; evidencing impact to funders; digital delivery of information in multiple languages; being consulted late by vendors; limited budgets.
Buying triggers	A funded programme needing digital delivery; a policy campaign needing evidence; an NHS partnership requiring co-design.
Decision maker	CEO / Director of Programmes; Head of Policy; trustees for larger commitments.
Budget (ESTIMATE)	Small: £3k–£15k for co-design or research work; more valuable as content and credibility partners than as customers; joint bids £20k–£100k.
Sales cycle	2–6 months.
Objections	"We do not endorse commercial products." · "Our content is our IP." · "Who benefits financially?"
Vytalix proposition (pillar)	Consulting (C8 listening exercises delivered <i>with</i> community organisations, paid); Health Solutions content and co-design partnerships (paid, never "endorsement"); E-Learning co-authored modules.

Attribute	Detail
Acquisition strategy	Approach with a paid brief, not a request for support; involve early in MaternaLink personas (/docs/04_MATERNALINK_PRODUCT_STRATEGY .md §3); co-design fee budgeted in every UK pilot.
Horizon	6m (partnership), revenue minor.

1.10 Universities and research institutions

Attribute	Detail
Who	Midwifery, nursing, public-health and health-informatics programmes; maternal-health research units (e.g., NPEU Oxford, King's, Manchester — institutions only); Nigerian and Kenyan universities/teaching hospitals; accelerators and incubators attached to universities.
Pain points	Industry-relevant teaching content on digital maternity; access to real-world data partners for research; impact and REF-style requirements; grant co-applicants from industry (Innovate UK/NIHR collaborative calls).
Buying triggers	Curriculum review; a funding call requiring an SME partner; an accelerator cohort needing mentors/workshops; a PhD studentship on digital maternity.
Decision maker	Programme lead / head of school (teaching); principal investigator (research); accelerator programme director.
Budget (ESTIMATE)	E-Learning university licence £3k-£8k per programme (TO VALIDATE); workshops £2.5k-£6k (T1); research partnerships are grant-funded (£50k-£500k shared).
Sales cycle	3-9 months (academic calendar).
Objections	"We need accredited/credit-bearing content." · "Co-authorship and data ownership?" · "Ethics approval timelines."
Vytalix proposition (pillar)	E-Learning (licences, co-authored modules); Advisory (accelerator workshops); Health Solutions research partnership for evaluation and the v3 AI track (no clinical claims).
Acquisition strategy	Offer guest lectures and accelerator workshops first (free or T1); identify two research groups for joint bids by month 9; sign data-partnership intent letters only after IP and ethics routes are clear.
Horizon	Workshops 6m ; licences 12m-18m ; research 12m+ .

1.11 Employers (corporate HR, DEI and wellbeing)

Attribute	Detail
Who	Large UK employers with maternity-return and women's-health programmes; benefits platforms (competition: Peppy, Maven's UK presence — TO VALIDATE); Nigerian/Kenyan corporates with employee health schemes.
Pain points	Retention of women through pregnancy and return; inclusive benefits for Black and minority-ethnic staff; fragmented benefits vendors; measuring wellbeing spend.
Buying triggers	Benefits renewal window (often Q4); DEI strategy launch; a high-profile maternity-related grievance; a gender-pay-gap report.
Decision maker	Head of Benefits / Reward; DEI lead; HR director; benefits broker/platform (gatekeeper).
Budget (ESTIMATE)	Education/workshops £5k-£15k (\$5 in /docs/13_SALES_ENGINE .md); product £2-£6 PEPM or £150-£300 per pregnancy (later).
Sales cycle	4-10 weeks (workshops); 6-12 months (benefit).
Objections	"Our platform already covers this." · "Clinical liability?" · "Utilisation will be low."
Vytalix proposition (pillar)	E-Learning/Consulting (women's-health programme design, manager education); Health Solutions non-clinical education and coordination via benefits platforms (24m+).
Acquisition strategy	Through benefits brokers and platforms, not direct to HR; one flagship webinar per quarter; opportunistic until Q2 2027.
Horizon	Workshops 6m ; product 24m+ .

1.12 Care organisations (community services, health visiting, social care, doulas)

Attribute	Detail
Who	Community health-visiting providers (NHS and local-authority commissioned); social-care teams involved in safeguarding in pregnancy; doula and peer-support organisations; in Africa, community health worker programmes (CHEWs, CHVs) run by states or NGOs.
Pain points	Handover from maternity to health visiting (the postnatal gap); safeguarding information flows; peer-support coordination; CHW supervision, caseload and defaulter tracing; paper-based reporting.
Buying triggers	A serious case review; a commissioning re-tender for 0-5 services; an NGO CHW programme grant; a state CHEW digitisation initiative.
Decision maker	Service lead for health visiting / 0-19 services; local-authority public-health commissioner; NGO programme manager; SPHCDA director.
Budget (ESTIMATE)	Small in the UK (£3k-£10k for design work); Africa CHW modules bundled into programme licences (£8k-£60k).
Sales cycle	3-9 months.
Objections	"Another app for staff who already have three." · "Information governance across organisations." · "Connectivity in the field."
Vytalix proposition (pillar)	Health Solutions (v2 postnatal handover and CHW workflow modules — offline-first design from /docs/07_TECHNOLOGY_ARCHITECTURE .md); E-Learning (CHW/health-visitor micro-courses).
Acquisition strategy	Ride on NGO and trust pilots; do not sell stand-alone before v2.
Horizon	18m-24m .

1.13 Health-tech companies (UK, inbound international, African)

Attribute	Detail
Who	UK digital-health start-ups and scale-ups (pre-seed to Series A); US/EU/African health-tech companies entering the NHS; UK/EU companies entering Nigeria/Kenya/Ghana; maternity EPR vendors (System C BadgerNet, K2, Magentus Euroking) seeking patient-facing partners; telehealth and messaging platforms.
Pain points	NHS is opaque (ICBs, DTAC, DCB0129, DSPT, procurement frameworks); investor-readiness and grant pipelines; evidence-generation planning; African market entry without local partners; regulatory classification uncertainty (MHRA SaMD).
Buying triggers	A funding round closed (cash available, a board asking for NHS traction); a rejected DTAC/procurement bid; an accelerator place; a US/EU company announcing UK ambitions; a grant deadline.
Decision maker	Founder/CEO or COO (decides within days); Head of Commercial; for larger vendors, VP Partnerships.
Budget (ESTIMATE)	Retainer £2.5k-£6k/month (A2); sprints C4 £22k, C5 £20k, C6 £15k, C7 £9.5k; first-year account value £9.5k-£40k.
Sales cycle	2-6 weeks (founders); 2-4 months (vendors with procurement).
Objections	"We can learn the NHS ourselves." · "What have you actually delivered in the NHS?" · "Conflict of interest with MaternaLink?"
Vytalix proposition (pillar)	Advisory (retainers, investor/grant readiness) and Consulting (C4 UK Market-Entry Playbook, C5 Africa Market-Entry & Partner Scan, C6 DTAC & Evidence Readiness, C7 Grant & Investor Readiness). Conflict rule: no advice to a direct MaternaLink competitor without written disclosure (/docs/01_BUSINESS_MODEL .md §5 catalogue rule d).
Acquisition strategy	LinkedIn outbound to founders who announced rounds in the last 90 days; accelerator and investor referrals; content on "NHS-ready in 12 months"; free 30-minute session as the hook (/docs/13_SALES_ENGINE .md §16).
Horizon	Now.

2. B2C segments

B2C segments are primarily *users* of MaternaLink (in development) and *learners* for E-Learning. Only E-Learning generates direct B2C revenue in the plan. Persona detail lives in /docs/04_MATERNALINK_PRODUCT_STRATEGY .md §3; this section covers the commercial and adoption logic.

2.1 Patients (general — the wider "Technology for Life" population)

Attribute	Detail
Who	Anyone receiving health care who experiences fragmented communication; the long-term vision population. Not a 2026-28 product segment.
Pain points	Not knowing who is responsible for their care; repeating their story; missed follow-ups.
Trigger to adopt	A provider offers the tool as part of care.
Decision maker	The provider (B2B2C); the patient consents.
Budget	£0 direct; provider pays.
Objections	Privacy; "another app"; language and literacy barriers.
Vytalix proposition (pillar)	Long-term Health Solutions vision only; no offer in this plan.
Acquisition	None; deferred.

2.2 Expectant mothers (UK and Nigeria/Kenya)

Attribute	Detail
Who	~568k births/year in England (ONS, above); Nigeria has the largest absolute maternal-death burden (IHME, https://www.healthdata.org/news-events/newsroom/news-releases/global-maternal-deaths-fell-240000-2023-more-100-countries-still); priority sub-groups: Black and Asian women in the UK (MBRRACE-UK disparity), women in deprived areas, women with limited English, rural women in Nigeria with feature phones.
Pain points	Not being listened to; changing professionals; unclear escalation ("who do I call?"); information in the wrong language or register; in Nigeria, cost and distance to facilities, and not knowing danger signs.
Trigger to adopt	Enrolment by a midwife/CHEW at booking or first ANC visit; a trusted community organisation recommending it; SMS/WhatsApp that works on the phone she already has.
Decision maker	The woman (consent and continued use); often influenced by partner/mother-in-law (Nigeria) and by the midwife's recommendation (UK).
Budget	£0 direct (provider/programme pays). Willingness to pay directly is low and is not modelled (ASSUMPTION; TO VALIDATE in discovery — 20 women per market per product strategy).
Objections	Data privacy; fear that messages replace human care; phone sharing within households; cost of data/SMS.
Vytalix proposition (pillar)	Health Solutions: MaternaLink (in development) — care plan visibility, named contacts, escalation guidance (non-diagnostic), multilingual education, appointment reminders.
Acquisition	Via the enrolling provider only; adoption metrics (activation, weekly use, escalation-guidance use) defined in /docs/04_MATERNALINK_PRODUCT_STRATEGY .md §24.
Horizon	Design partner 12m ; pilots 18m+ .

2.3 Families (partners, birth partners, mothers-in-law, carers)

Attribute	Detail
Who	The people who make decisions with or for the woman, particularly in Nigeria where household decision-makers affect care-seeking; in the UK, partners who want to help and are often excluded from communication.
Pain points	Excluded from information; unclear on danger signs; unable to reach the care team; cost decisions made without information.
Trigger to adopt	The woman shares access; a CHEW/midwife enrolls the household contact.
Decision maker	The woman (delegated access); safeguarding rules apply (coercive-control risk — /docs/04_MATERNALINK_PRODUCT_STRATEGY.md §19).
Budget	£0.
Objections	Privacy and safety (a shared account can be a risk); confusion about who is the patient.
Vytalix proposition (pillar)	Health Solutions v2 delegated-access feature with safeguarding controls; E-Learning free primers.
Acquisition	With the woman's enrolment; never marketed independently.
Horizon	v2 (months 7–15 after MVP).

2.4 Healthcare learners (students, newly qualified, career-changers into health-tech)

Attribute	Detail
Who	Midwifery, nursing and public-health students; health-informatics students; early-career professionals in the UK, Nigeria, Kenya, Ghana; aspiring health-tech founders.
Pain points	Curricula lag digital practice; no practical content on digital maternity, data protection, safeguarding online; scarce, expensive CPD; no bridge from clinical to health-tech careers.
Trigger to buy	Course start, placement, job application, exam; a university embedding the course; a scholarship or cohort discount.
Decision maker	The learner (self-serve), or the university/employer (licence).
Budget (ESTIMATE)	£49–£149 per course (UK); £10–£40 regional pricing (Africa); £19/month subscription (/docs/01_BUSINESS_MODEL.md §6).
Objections	"Is it accredited?" (Vytalix holds no accreditation — must not be claimed); "Will my employer recognise it?"; price relative to free content.
Vytalix proposition (pillar)	E-Learning : courses 3 and 4 (care coordination in low-resource settings; grants and investor readiness) first; certificate of completion only.
Acquisition	Content marketing, LinkedIn, university partnerships, student ambassadors; CAC target < £60 (/docs/01_BUSINESS_MODEL.md §8 #8).
Horizon	6m–12m.

2.5 Healthcare professionals (midwives, obstetricians, nurses, CHEWs, digital leads, health-tech staff)

Attribute	Detail
Who	~UK midwifery and obstetric workforce (TO VALIDATE numbers from NMC/RCM sources); Nigerian and Kenyan nurses, midwives and CHEWs; digital midwives and maternity digital leads; health-tech commercial staff.
Pain points	Digital tools imposed without training; documentation burden; handover risk; revalidation/CPD hours; wanting to influence the tools they use.
Trigger to buy	Revalidation cycle; a new system going live; a promotion into a digital role; an employer licence.
Decision maker	Individual (self-serve) or Head of Midwifery/L&D (corporate licence).
Budget (ESTIMATE)	£99–£299 per course; corporate licence £2.5k (10 seats) to £25k (250 seats); workshops £2.5k–£6k.
Objections	Accreditation; time; "our trust has its own training"; relevance of UK content in Nigeria (localisation required).
Vytalix proposition (pillar)	E-Learning (Digital Maternity Essentials; digital-midwife pathway — clinical co-author required); Health Solutions (professionals are the daily users of the clinician view and the source of design-partner credibility).
Acquisition	Professional networks and digital-midwife communities (no endorsement claimed); conference workshops; every consulting engagement produces at least one reusable learning asset.
Horizon	Workshops 6m ; courses 12m–15m.

3. ICP ranking

3.1 Scoring model (PROPOSED)

Six criteria, weights sum to 100; each scored 1–5 (5 = most favourable); ICP score = $\Sigma(\text{weight} \times \text{score})/5$.

Criterion	Weight	What "5" means
Speed to cash (sales cycle, procurement)	25	Signs in < 6 weeks without procurement
Budget accessibility (first-engagement value vs effort)	20	£10k–£40k obtainable from one decision maker

Criterion	Weight	What "5" means
Founder reach (can a solo founder access the decision maker now?)	20	Direct, founder-to-founder or via LinkedIn/referral
Strategic value to MaternaLink (learning, evidence, references)	15	Directly shapes or de-risks the product or its route to market
Competitive intensity (room for a boutique)	10	Few credible alternatives at Vytalix's price point
Regulatory/compliance burden to serve	10	No patient data, no assurance artefacts needed

3.2 Ranked B2B segments (desk ESTIMATE; re-score after 40 conversations)

Rank	Segment	Speed (25)	Budget (20)	Reach (20)	Strategic (15)	Competition (10)	Reg. burden (10)	ICP /100	Pillar lead	Horizon
1	Health-tech companies (UK + inbound + African)	5	4	5	4	3	5	89	Advisory / Consulting	Now
2	NGOs and development organisations	3	4	3	5	3	4	72	Consulting → Health Solutions	6m
3	Universities, accelerators, incubators	3	2	4	4	4	5	69	E-Learning / Advisory	6m
4	Clinics and private maternity units	4	2	3	5	4	3	69	Health Solutions / Consulting	6m–12m
5	Charities and community organisations	3	1	4	5	4	5	68	Consulting / partnership	6m
6	Healthcare groups (private, multi-site)	3	4	3	3	2	4	64	Consulting	6m–12m
7	Governments — UK (ICB/LMNS, local authority)	2	4	2	4	2	3	56	Consulting	12m
8	Employers and benefits platforms	3	2	3	2	2	5	55	E-Learning / Consulting	6m (workshops)
9	Governments — African	1	3	3	5	2	2	52	Advisory → Health Solutions	12m / 24m+
10	Pharma (global health / medical affairs)	1	4	2	3	2	4	50	Consulting	12m–24m
11	Hospitals / NHS trusts	1	3	2	5	2	1	46	Consulting → Health Solutions	12m / 24m+
12	Insurers, HMOs, benefits platforms	1	3	2	3	3	2	44	Health Solutions	24m+
13	Care organisations (community, CHW programmes)	2	1	2	3	4	2	43	Health Solutions v2	18m–24m

Reading the table: the top three segments are the only ones where a solo founder without an entity, references or assurance artefacts can close in 2026. Segments 4–7 open once incorporation, PI insurance and one or two case studies exist. Segments 8–13 are 2027–28 revenue and 2026 relationship-building. The **time-allocation rule** in [/docs/13_SALES_ENGINE.md](#) (≥60% of selling time on health-tech companies and investors/accelerators in the first 90 days) follows directly from this ranking.

3.3 ICP statement for the services business (PROPOSED)

A digital-health company — UK-based or entering the UK or an African market — between pre-seed and Series A, with 5–50 staff, that has raised or is raising money in the last or next six months, whose founder or COO owns the go-to-market problem (NHS entry, assurance, evidence, grants, investor narrative, or Nigeria/Kenya entry), and who can sign a £7.5k–£22k fixed-price engagement or a £2.5k–£6k monthly retainer without a procurement process.

Disqualifiers: a direct MaternaLink competitor without disclosure; companies wanting equity-only arrangements; companies whose product makes clinical claims Vytalix would have to endorse.

3.4 ICP for MaternaLink (in development) — design partner, not customer (PROPOSED)

A UK NHS trust maternity service with a digital midwife in post, an active improvement plan (CQC, national investigation or Ockenden-type actions), a Clinical Safety Officer willing to run DCB0160, and one community midwifery team that will co-design and evaluate a non-diagnostic communication and handover tool for 3–6 months at no licence fee; or an African NGO with an enrolled cohort of ≥2,000 women, CHEWs under supervision, donor reporting obligations and an evaluation deadline within 18 months.

4. First ten target-account archetypes for the services business

Archetypes, not named accounts. Each row states the publicly observable trigger that puts an account into the CRM, the entry offer, the decision maker and where such accounts are found. Named accounts are entered in the CRM only from public sources (Companies House, press releases, LinkedIn company

pages, grant award lists) and never with invented contact details (FACTS_BASE \$4).

#	Archetype	Observable trigger (last 90 days)	Decision maker	Entry offer (price ESTIMATE)	Why they buy from a new boutique	Where to find them	90-day target
A1	Recently funded UK digital-health start-up (pre-seed/seed, £0.5m–£5m raised) with NHS ambitions and no NHS customer	Funding announcement; job ads for "NHS partnerships"; accelerator demo day	Founder/CEO	C7 Grant & Investor Readiness (£9.5k) or A2 retainer (£4k/month) → C6 DTAC & Evidence Readiness (£15k)	Founder-to-founder; fixed price; they have cash and a board asking for NHS traction	UK health-tech funding trackers, accelerator cohort pages, LinkedIn	15 contacted, 5 calls, 2 proposals, 1 signed
A2	US or EU health-tech company announcing UK/NHS entry (Series A–B)	Press release "expanding to the UK"; UK entity registered at Companies House; first UK hire posted	VP International / UK Country Lead / COO	C4 UK Market-Entry Playbook (£22k)	They need an NHS map and assurance checklist faster than a big firm will deliver it	Press releases, Companies House new incorporations with foreign parent, LinkedIn job posts	10 contacted, 3 calls, 1 proposal
A3	UK/EU health-tech or NGO planning Nigeria/Kenya/Ghana entry	Announcement of an Africa pilot; a donor grant for an African programme; hiring a "Head of Africa"	CEO / Head of International / Programme Director	C5 Africa Market-Entry & Partner Scan (£20k)	Vytalix's UK–Africa corridor positioning (usable once founder credentials support it — TO VALIDATE)	Grant award lists (Innovate UK global health, GCC, Gates), press, LinkedIn	8 contacted, 3 calls, 1 proposal
A4	Health-tech accelerator, incubator or investor programme running a cohort in the next 6 months	Cohort application open; demo day scheduled; mentor call-out	Programme Director / Partner	Cohort workshop (T1 £4k) or programme package (£5k–£20k)	Fills a gap (NHS-readiness, Africa entry, grants) with a practitioner; low risk to them	Accelerator websites, university innovation centres, investor portfolio pages	8 contacted, 3 calls, 1 workshop booked
A5	Implementing NGO with a maternal/digital programme grant awarded in 2026 in Nigeria or Kenya	Grant announcement; RFP for a technology or evaluation partner; job ad for a digital health officer	Country Director / Programme Manager / M&E lead	Programme design or Evaluation Lite (£10k–£26k); joint bid	They must spend against a logframe and need a partner who understands both digital and maternal care	Donor award databases, NGO newsrooms, Devex-type listings	8 contacted, 3 calls, 1 proposal
A6	Private maternity unit or obstetric group launching or refreshing a service	Refurbishment/launch press; new lead consultant; patient-experience campaign	Hospital Director / Lead Obstetrician	C1 Discovery Sprint (£14k) → MaternaLink design conversation	Fast decisions; differentiation motive; fewer IG barriers	Private hospital newsrooms, LinkedIn, Care Quality Commission registrations	6 contacted, 2 calls, 1 proposal
A7	NHS trust with a new Director of Midwifery or a recent maternity CQC report (introductions only in 2026)	Appointment announcement; CQC report; national investigation listing	Director of Midwifery; Maternity Digital Lead	Free 30-minute discovery; C3 Readiness Review (£24k) or C8 Inequalities Review (£28k) via sub-contract	Vendor-neutral boutique with a clinical adviser; sub-contracted through a prime for procurement	NHS trust news pages, CQC reports, Health Innovation Network events	10 contacted, 4 discovery interviews, 0–1 proposal
A8	LMNS/ICB with an equity or digital maternity workflow and a published plan	LMNS equity plan; board paper on maternity; MNVP call for partners	LMNS Programme Director; Health-Inequalities Lead	C8 Health Inequalities & Maternity Outcomes Review (£28k) or C2 Business Case (£18k)	Evidence-based, community-partnered review at boutique price	ICB board papers (public), LMNS pages, MNVP networks	8 contacted, 3 interviews, 1 proposal (2027 close)
A9	Maternity EPR or telehealth vendor seeking patient-facing/communication partners	Product roadmap announcement; partnership call; a trust go-live press release	VP Partnerships / Product Director	Advisory retainer (£4k/month) on patient-facing strategy; later a MaternaLink integration conversation (v2)	Vytalix is small enough to partner with and understands the maternity workflow	Vendor press (System C, K2, Magentus — institutions only), Digital Health news	5 contacted, 2 calls
A10	University maternal-health research group or midwifery programme preparing a grant or curriculum refresh	Funding call open (NIHR, UKRI, Wellcome); curriculum consultation; PhD studentship advert	Principal Investigator / Programme Lead	Guest lecture (free) → T1 workshop (£4k) → joint bid co-applicant	An SME partner strengthens bids; practical content strengthens curricula	University research pages, NIHR/UKRI call calendars, LinkedIn	6 contacted, 3 calls, 1 joint-bid intent

Aggregate 90-day target across archetypes (PROPOSED): ~85 contacted, ~30 conversations, ~10 proposals, 3 signed — consistent with [/docs/13_SALES_ENGINE.md](#) (150 qualified contacts, 40 discovery calls, 12 proposals, 3 signed) once investor and warm-intro channels are added.

4.1 Archetype qualification checklist (use on every account before it enters stage 1)

1. Trigger observed in a public source within 90 days (URL recorded in CRM).
2. Decision-maker title identified from a public source; no invented emails or phones.
3. Budget plausibility: raised money, holds a grant, or has a published programme budget.
4. No undisclosed conflict with MaternaLink (in development).
5. Entry offer chosen from the catalogue; no bespoke scope before a discovery call.
6. Next action dated.

5. Segment × pillar × offer map and 12-month segment targets (PROPOSED)

Segment	Advisory	Consulting	Health Solutions (MaternaLink, in development)	E-Learning	12-month revenue target (ESTIMATE)	12-month non-revenue target
Health-tech companies	A1 days, A2 retainers	C4, C5, C6, C7	— (integration conversations with vendors)	Founder course (course 2)	£50k–£90k	2 case studies (with consent)
NGOs / development	Programme design	Evaluation Lite (#10), C5	Programme licence scoping; joint bid	Co-branded modules (later)	£10k–£30k	1 joint grant bid; 1 cohort LOI
Universities / accelerators	Workshops	—	Research partnership intent	Licences (later)	£5k–£15k	1 co-applicant agreement
Clinics / private maternity	—	C1	Design conversation; paid pilot (12m+)	—	£5k–£14k	1 pilot LOI
Healthcare groups	Fractional CSO (#9)	C1, C3	—	Corporate licence (later)	£0–£28k	1 discovery relationship
Charities / community	—	C8 co-delivery (paid to them)	Co-design partnership	Co-authored content	£0–£5k	2 paid co-design agreements
Employers	—	Programme design	—	Workshops	£0–£8k	1 webinar
Governments UK	—	C2, C8 via prime	—	—	£0–£28k	1 sub-contract relationship
NHS trusts	—	C3, C8	Design partner (unpaid)	Workshops	£0–£24k	15 discovery interviews; 1 design partner
Pharma	—	Evaluation, landscape	Funded cohort (later)	—	£0	1 programme-officer conversation
Governments Africa	Programme design (donor-funded)	—	Partner-aware pilot scope	—	£0–£15k	Ekiti status resolved; mDoc conversation; 2 states scored
Care organisations	—	—	v2 scoping	Micro-courses (later)	£0	Interviews for v2
Insurers / HMOs	—	—	Discovery interviews	—	£0	5 interviews
Total					£70k–£210k (bounds consistent with /docs/01_BUSINESS_MODEL.md §4: base £118k)	

6. Discovery questions by segment (add to /docs/13_SALES_ENGINE.md §5 question bank)

Segment	Three questions that qualify or kill
Health-tech company	What did your last NHS/African conversation stall on? What does your board expect by the next round? Who signs a £10k–£20k engagement and how fast?
NGO	Which grant funds this and when does it end? What does your M&E lead need that your current tools do not give? Who maintains technology after the project?
NHS trust	Which CQC/investigation action is unresolved and owned by whom? Who is your Clinical Safety Officer and have they run a DCB0160 for a patient-facing tool? Which EPR and what is its contract end date?
Private maternity	How do women reach the team today between visits? What differentiates you from the NHS in the client's eyes? What would a pilot need to prove, and to whom?
African government/donor	Who funds digital maternal programmes in this state today? What happened to the last vendor proposal? What data must the state own and where must it be hosted?
University	Which call are you preparing for and does it need an SME partner? Who owns data and authorship? What ethics route applies?

Priorities / Risks / Next actions

Priorities

1. Sell only to ICP rank 1–3 for the first 90 days (health-tech companies, NGOs, universities/accelerators); treat NHS, government and pharma as interview segments until an entity, insurance and one case study exist.
2. Load the CRM with 85 archetype-matched accounts from public triggers (A1–A10) by day 14; record the trigger URL for each.
3. Run 15 UK NHS/private maternity discovery interviews and 10 Nigeria/Kenya NGO/donor interviews by day 60 to replace the ESTIMATE budgets and cycles in §1 with observed data.
4. Publish the ICP statement and the ten-offer catalogue on the website (once entity details are known) so inbound is pre-qualified.
5. Re-score §3.2 at day 90 with observed conversion rates.

Risks

- Price-point inconsistency: `/docs/01_BUSINESS_MODEL.md` (founder day £1,200–£1,800) and `/docs/13_SALES_ENGINE.md` (£900 list, £800–£1,200 range) quote different day rates; proposals must use one list (`/assets/PRICING_STRUCTURE.md`) — decision needed before the first proposal.
- Segment drift: the founder spends time on the Ekiti government conversation (emotionally live, low ICP score) at the expense of health-tech outbound. Mitigation: time rule in `/docs/13_SALES_ENGINE.md` §1.
- Conflict of interest: advising health-tech companies that later compete with MaternaLink (in development). Mitigation: disclosure clause and conflicts register.
- Over-claiming corridor expertise ("worked on both sides of the UK–Africa corridor") before the founder's credentials and first engagements support it (TO VALIDATE against the founder's CV).
- B2C expectations: investors may ask for consumer traction; the plan deliberately has none — say so.

Next actions

- Day 1–7: agree the single price list; write the ICP one-pager; build the A1–A4 account lists (40 accounts) from public sources.
- Day 7–14: build A5–A10 lists (45 accounts); draft segment-specific first messages using `/docs/13_SALES_ENGINE.md` §12–13 templates.
- Day 14–60: interviews (25) and outbound per \$4 targets; log objections verbatim in the CRM.
- Day 60–90: first re-score of \$3.2; decide whether private maternity or NGOs become the MaternaLink design-partner route; update `/docs/12_PARTNERSHIP_STRATEGY.md` accordingly.